

Restraint and Seclusion Policies

Strict Guidelines for Using Restraints

Regulation: 42 CFR §482.13(e)(f)

POLICY OVERVIEW

Restraint and seclusion are interventions of last resort that carry significant risk of patient harm and death. Hospitals must have comprehensive policies that minimize their use and ensure appropriate oversight when they are employed.

KEY REQUIREMENTS

1 Least Restrictive Alternatives

Before applying restraints or seclusion, staff must attempt and document less restrictive interventions. The patient's behavior must represent a clear and present danger to self or others that cannot be managed by less restrictive means.

2 Order Requirements

Restraints require a physician or licensed independent practitioner order. Orders must specify: type of restraint, clinical justification, and time limit (4 hours for adults, 2 hours for ages 9-17, 1 hour for under age 9). Telephone orders must be confirmed in writing within 24 hours.

3 Monitoring Requirements

Patients in restraints must have continuous assessment. Monitoring includes: vital signs, circulation/sensation in restrained extremities, skin integrity, nutrition/hydration, elimination, psychological status, and readiness for restraint removal.

4 Behavioral Health Restraints

Additional requirements apply for restraints/seclusion used for behavioral management: face-to-face evaluation within 1 hour of initiation by a physician or trained RN/PA, and more frequent monitoring.

5 Documentation & Reporting

All restraint use must be thoroughly documented. Deaths or serious injuries occurring while in restraints or within 24 hours of removal must be reported to CMS within 24 hours.

COMPLIANCE NOTES

Restraint and seclusion policies must distinguish between medical/surgical restraints and behavioral health restraints, as different standards apply. Staff must receive competency-based training before applying restraints.

COMMON SURVEY CITATIONS

Heavily cited under §482.13(e) – Safe environment, and §482.13(f) – Restraint or seclusion standards.